



SCGOPHCG Clinical Practice Guideline

Identification, Escalation and Management of Behavioural Disturbances (BD)

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All clinical staff

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Introduction

The Sir Charles Gairdner Osborne Park Healthcare Group (SCGOPHCG) is committed to providing healthcare that minimises the risk of physical and psychological harm to staff and patients. The early identification, escalation and individualised engagement plans to care for patients exhibiting acute / acute severe behavioural disturbances (ABD / ASBD) or behaviours of concern (BoC) is crucial to ensuring positive patient experiences and the wellbeing of staff.

This Clinical Practice Guideline aims to provide staff guidance on recognising and responding to changes in cognition, behaviour and mental health in patients at SCGOPHCG and is aligned with the Australian Commission on Safety and Quality in Health Care Standards. Please see related documents for reference to these standards and consensus statements.

This Clinical Practice Guideline should be read in conjunction with the [SCGOPHCG Cognitive Impairment Management \(Delirium and/or Dementia\) Clinical Practice Guideline](#).

Emergency department “[did not wait](#)” or “left at own risk” are out of scope for this guideline.

Risk Statement

Non-compliance with this Clinical Practice Guideline will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☒
Other:	☐		

Definitions

Behavioural Disturbance (BD)	Can be used interchangeably with Acute Severe Behavioural Disturbance (ASBD) Acute Behavioural Disturbance (ABD) and/or Behaviours of Concern (BoC) and puts the patient or others at risk of significant harm. Behaviours can present as positive symptoms for example (but not exhaustive). - Distress, confusion, agitation, aggression, erratic behaviour, delusions, hallucination, paranoia, disorganised speech. Or negative symptoms (not exhaustive); - Suicidal, self harm, withdrawn, uncommunicative, ambivalent regarding treatment, flat affect.
Behavioural and Psychological Symptoms of Dementia (BPSD)	Symptoms of unpredictable altered perception, thought content, mood, behaviour, and sleep disturbances that occur in patients with dementia

Cognitive Impairment (CI)	An overarching term referring to deficits in one or more of the areas of memory, problems with communication, attention, thinking and judgement. Dementia and delirium are common forms of cognitive impairment seen in older hospitalised patients. ¹ Other causes include stroke, acquired brain injury, depression, and intellectual disability.
Delirium	An acute change in mental status characterised by a disturbance in consciousness, attention, cognition, and perception that develops over a short period of time usually over hours or days. Delirium is a serious condition and should be treated promptly and appropriately.
Dementia	A progressive decline in cognitive function that affects memory, judgement, attention, language, and problem solving. It is usually gradual, progressive, and irreversible, leading to impaired functioning.
Discharge Against Medical Advice (DAMA)	Defined by the WA Health Patient Activity Data Policy (MP 0164/21) Admitted Patient Activity Data Business Rule . Discharged Against Medical Advice' (DAMA) occurs when a patient chooses to leave the hospital before the completion of treatment against the advice of the treating medical practitioner.
Four A's Test (4AT)	A brief screening tool for the detection of possible delirium as well as cognitive impairment
Mental Health Disorder	Is characterised by a clinically significant disturbance in an individual's cognition, emotional regulation, or behaviour. Usually associated with distress or impairment in important areas of functioning or risk of self-harm.
Patient Initiated or Supported Discharge	An alternative discharge plan agreed upon by the treating team and the patient and/or next of kin, guardian, or carer.
Behaviour Based Alert (BBA) - webPAS / EDIS / PSOLIS	Is an alert placed in one of the patient administration systems to identify previous and/or ongoing behavioural risk that puts the patient, staff or others at risk of significant harm. A governance process will support the maintenance of the BBA. webPAS – Patient Administration System: State hospital patient management solution. EDIS – Emergency Department Information System: Both a workflow and data collection tool capturing real-time information about patients and to support the operational control of WA Emergency Departments. PSOLIS – Psychiatric Services On-Line Information System: Clinical information and patient management system.

General Principles

Early identification of patients at risk of BD is essential for proactive, preventative, and patient centred care. The principles underpinning every interaction and management plan should be:

- Trauma informed.
- Multidisciplinary
- Least restrictive
- Culturally competent and sensitive

Behaviours of concern and behavioural disturbance can be identified in 3 ways:

- **Screening tool:** using Patient Admission Assessment Form (PAAF), previous alerts and patient history.

- **Trigger tool:** Page 1 of the BoC escalation pathway, page 2 of the pathway guides initial assessment and management)
- **Monitoring and Escalation tool:** Behavioural Observation Form (BOF)

High risk / vulnerable patients identified on the screening tool may be placed pre-emptively on the BOF to monitor for deterioration much like the Observation Response Chart (ORC).

Clinicians, family, or carers may become concerned about a previously well patient, whose cognition, behaviour, or mental health has changed. This would warrant the use of a trigger tool (BoC escalation pathway), to identify the causes and direct further management, including ongoing monitoring using the BOF.

Likewise, escalations on the BOF, may warrant further investigation and management using the BoC escalation pathway, medical review, referral to Behavioural Evaluation and Review Team (BERT), Urgent BERT (SCGH only) or code black.

If at any time any staff member feels threatened or unsafe, activate a code black by calling “55” immediately.

1.0 Early Identification of Changing Behaviours

ASBD/ABD/BoC are umbrella terms that identify changes in cognition as well as positive and negative changes to alertness and behaviour. The causes of these symptoms can be due to cognitive impairment (eg. developmental, acquired, dementia), delirium (medical eg. sepsis, toxicological) and mental health issues. Furthermore, a change in behaviour may be caused by one or more aetiologies. The table below highlights contributing factors to BD and early warning signs, these are not exhaustive:

Contributing Factors	Early warning Signs
• Acute mental health disorders • Dementia +/- BPSD • Delirium • Acquired or traumatic brain injuries • Neurological conditions • Neurodevelopmental impairments, including, Foetal Alcohol Spectrum Disorder (FASD) • Post anaesthesia/extubation • Pain • Alcohol / substance misuse / withdrawal • Grief and loss • Fear, anxiety and distress • Unexplained long waiting periods • Lack of Communication • Infection • Sleep disturbances • Poor cultural awareness	• Agitation or restlessness • Increased respirations, heart rate and sweating • Confusion • Ambivalence towards treatment • Withdrawn/uncommunicative. • Clenched jaw, hands, facial or body muscles • Pacing • Increase volume of speech, erratic movements. • Unclear thought process, delusions, or hallucinations • Fear, inability to communicate. • Reporting anger or distress • Replication of previous behaviours

Early identification of a change in cognition, behaviour, or mental state should be occurring on a regular and ongoing basis for all patients regardless of risk or predisposition to develop a BD. Patients could have preexisting aggression alerts in webPAS, EDIS or PSOLIS and be admitted with BD or they may develop during their admission.

2.0 Initial Assessment

1. Cognitive Assessment

Cognition is assessed using the **4AT**. The 4AT is a rapid clinical test which has been validated both for screening for cognitive impairment and delirium.¹ It should be used in conjunction with presenting diagnosis and behaviours, past medical history and any behaviour based alert notification when considering further escalation.

The 4AT has 4 items, each of which begins with an 'A':²

- 1) **A**lertness
- 2) **A**MT4: Abbreviated Mental Test 4
- 3) **A**ttention: Month of the Year Backwards
- 4) **A**cute change or fluctuating course

The 4AT is used in both the screening (Patient Admission Assessment Form - PAAF) and page 1 of the BoC escalation pathway trigger tool. The PAAF must be completed within 8 hours of admission to hospital and is one of the earliest identifiers of baseline cognition.

*For more information on how to conduct **4AT**, or access to 4AT in languages other than English, visit: [4AT - Rapid Clinical Test for Delirium Detection](#)*

- A 4AT score of 1-3 suggests possible cognitive impairment.
- A score of 4 or higher is suggestive of delirium with or without cognitive impairment.

The new concerns may be identified by:

- Reported changes by patient, family, next of kin and/or carers,
- Escalation on BOF, or
- when dual (organic and mental health) pathologies **cannot** be excluded / are suspected.

Any unexplained change in 4AT should prompt further medical assessment and management, by the treating team, as outlined on page 2 of the BoC escalation pathway. After hours, the patient should be reviewed by the on call medical officer and handed over to treating team the following day. A **Behavioural Observation Form (BOF)** should be commenced to monitor ongoing changes in behaviour and cognition.

*The BoC escalation pathway is **not** suitable if the patient is being admitted for the purpose of investigating and managing an isolated delirium or mental health disorder.*

2. Intensive Care Cognitive Assessment

The Intensive Care Unit (ICU) use the Richmond Agitation-Sedation Scale (RASS) to assess if the patient is suitable for delirium screening. RASS is recorded hourly and if a patient score is ≥ -3 , the Confusion Assessment Method for the ICU (CAM-ICU) is completed twice daily.

3. Behaviour Assessment

The behaviour assessment at the bottom of page 1 of the BD Pathway is designed to capture potential mental health aetiologies causing changes to behaviour, alertness, or cognition.

If a patient answers yes to any of the below **AND** there is a **HIGH SUSPICION** of mental illness driving these behaviours, please escalate to a senior medical officer for consideration of referral to Psychiatry.

Behaviours of Concern	Concern about patient actions or behaviour		
	Aggressive, threatening or violent behaviour	No	Yes
	Absconding behaviour	No	Yes
	Conversation illogical and hard to follow	No	Yes
	Hallucinations or delusions	No	Yes
	Concerns about mood	No	Yes
	Self-harm or suicidal behaviour	No	Yes
	Others (specify)	No	Yes
If ANY concerns about behaviours proceed to page 2, consider initiating Behavioural Observation Form (BoF) If ANY concerns about behaviours AND there is HIGH SUSPICION that this is driven by mental illness, consider early escalation to senior medical officer and referral to Psychiatry Psychiatry / Mental Health Referral ☐			

4. Behavioural Observation Form (BOF)

A Behavioural Observation Form should be commenced in any of the following conditions:

- A high-risk patient is flagged by the Patient Admission Assessment form (PAAF) screening tool.
- Behavioural monitoring is required after the BoC escalation pathway has been activated.
- Whenever a perceived or actual risk of escalation in cognitive, behavioural, or mental health..

The BOF will demonstrate patterns of behaviour, to aid in the identification, escalation, intervention, and engagement plans of these complex patients. There is no prescribed time to cease recording changes in cognition, behaviour or mental health, however a return to baseline should be established for at least 72 hours. Should behaviours re-emerge, the BOF should be recommenced.

5. Patient Initiated or Supported Discharge or Discharge Against Medical Advice (DAMA)

Some patients with behavioural disturbance may threaten to take their own discharge. If a patient wishes to leave the hospital before their treatment has finished staff should refer to the [SCGOPHCG Detention of Patients and Discharge Against Medical Advice Policy](#). The policy outlines:

- Capacity and risk assessment.
- Patient Initiated or Supported discharge.
- Discharge without agreement from treating team (DAMA) and absconding.

3.0 Nursing Assessment and Management

1. Communication

- Staff should be aware of communication barriers, such as hearing impairment, dysphasia, language barriers and cultural competence when engaging with patients and their support people.
- Interpreters, Patient Liaison, Aboriginal Health Liaison Officers, and Allied Health should be engaged when required.
- BERT CNS have access to KP App for interpreting services.
- When delays in care provision or unforeseen circumstances occur, this should be communicated with patients, family, next of kin and/or carers.
- Patient communications plans need to be provided within management and or care plan.

2. Prevention Strategies

A Behavioural Observation Form (BOF) should be initiated if a patient has a new change in behaviour, has a history of behavioural changes, or is at risk of exhibiting BD. The BOF is used for the monitoring, early identification of escalation in behaviour and implementing positive engagement strategies to enable de-escalation.

- Complete the initial behaviour assessment on the front of the BOF.
- Where possible engage with the patient, family, next of kin and/or carers, to determine history and triggers for BD as well as known prevention, de-escalation, or management strategies already in place.
- Patient and family preference should be considered when managing BD in the least restrictive way.
- Consider if the environment is safe and conducive to the ongoing care of the patient.
- Refer to [SCGOPHCG Cognitive Impairment Management \(Delirium and/or Dementia\) Clinical Practice Guideline](#) for specific prevention and intervention strategies.
- Consideration for a [Behaviour Evaluation and Review Team \(BERT\)](#) review. This can be requested through eReferral, email or phone.
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3. Escalation of Change in Cognition, Behaviour or Mental Health

The BOF provides guidance on the escalation and observation of patients with a history of, a change in, or at risk of exhibiting acute severe behavioural disturbance or behaviours of concern. This includes:

- An increased visual observation guideline and clinical escalation pathway through Shift Coordinators, Senior Registered Nurses, and Medical review.
- Physiological observations should only be undertaken if patient is agreeing to the same and /or it is safe to do so according to their clinical need.
- Consider commencing the BoC escalation pathway.
- Provides an area to describe situation, triggers, prevention strategies and escalation taken.
- **** For SCGH only** URGENT BERT call 75021. A BERT CNS will attend within **10 minutes** to assist with de-escalation.**
- For patients located off the SCGH site please email the [SCGH, BERT](#) or phoning the BERT Coordinator of Nursing.

At any stage if staff feel unsafe, a code black is to be initiated by dialling “55”

4. Management Strategies

- Ideally, prevention strategies that have been previously discussed are in place and in line with patient, family next of kin and/or carer preference.
- Attempt to re-orientate patient and use non-pharmacological strategies to deescalate the behaviours exhibited.
- URGENT BERT (SCGH only) call 75021 to assist with de-escalation.
- Pharmacological management can be offered if other strategies have failed, an oral option should be provided as a first line.
- If there is no pharmacological option a medical review should occur, ideally by the treating team or specialist consulting team, to determine the right prescription for the patient.

- Consideration of a clinical companion, as per the [Clinical Companion and Patient Special Management Policy](#).
- Supported early discharge is an option if consultation has occurred between treating teams and family, next of kin or carers and where appropriate Residential Care Line.
- If restrictive practice is required, the least restrictive option for the least amount of time should be used, this includes pharmacological and mechanical. Please refer to [SCGOPHCG Safe Use of Restrictive Practices in a Non-Authorised Setting](#).
- Ensure restrictive practices are communicated with the next of kin and entered on the restraint status form.
- Consideration of adding a Behaviour Based Alert (BBA)

4.0 Medical Assessment and Management

A medical review can be initiated for one or more of the following:

1. Patient Admission Assessment Form (PAAF) which includes:
 - 4AT assessment.
 - Previous mental health history.
 - Contact with mental health services or MHA 2014 status.
 - Changes in mental health state.
 - Changes in behaviour.
 - Identified wellbeing needs or risks.
 - Guardianship orders.
2. Deterioration or escalation identified using the BOF.
3. Unexpected change in cognition, behaviour and mental health identified using the BoC escalation pathway.
4. Concerned raised from a family member, significant other or carer around a change in cognition, mental health, or behaviour.

Ideally the treating medical team or the Charlies After-hours Team (CAT) should review the patient, identify triggers, perform necessary investigations to inform ongoing management plans.

1. Possible Triggers and Investigations

Organic pathology should always be considered **before** attributing any changes in cognition mental health or behaviour to an underlying mental health disorder. The second page of the BoC escalation pathway outlines the possible triggers and investigations that could be considered and performed at any time to help inform the treating and consultation teams.

	Ix	Aetiology for BOC	Potential Investigations (circle as required)	Other
Common Triggers	☐	Urinary retention	Bladder scan	
	☐	Constipation	Stool chart	Refer to MR885
	☐	Pain	Ensure adequate analgesia. Investigate further as required	
	☐	Hypoglycemia	Blood glucose	Refer to MR846
	☐	Hypoxia and Hypercapnia	Oxygen saturations. VBG or ABG. CXR	
Acute organic illness	☐	Sepsis	Septic screen - Urinalysis / Urine m/c/s, CXR, Blood cultures	Sepsis Pathway MR 878
	☐	Metabolic disturbance	Check UECs / Ca / Mg / Phos / TFTs / nutritional screen	
	☐	Acute coronary syndrome	ECG / Troponin	Call Cardiology
	☐	Acute neurological event	CT head / angiogram	Code STROKE
	☐	Other acute pathology	Consider other medical comorbidity or surgical illness	
Drug withdrawal	☐	Drug toxicity / adverse reaction	Review new / recent medication changes	
	☐	Alcohol withdrawal	Refer to appropriate withdrawal forms available for alcohol, cannabis, benzodiazepines and methamphetamines	Refer to MR410
	☐	Nicotine withdrawal		
	☐	Other substances	Refer to Alcohol and Drug services ☐	

Behavioural disturbances should only be attributed to a Mental Health Disorder after organic causes are excluded									
Mental Health	PHQ-4 Questionnaire for Anxiety and Depression ¹				Not at all	Several days	More than half the days	Nearly every day	Score
	Over the last 2 weeks, how often have you been bothered by the following problems?								
	Feeling nervous, anxious or on edge				0	1	2	3	
	Not being able to stop or control worrying				0	1	2	3	
	Feeling down, depressed or hopeless				0	1	2	3	
	Little interest or pleasure in doing things				0	1	2	3	
	Psychological distress, Severe (Score 9-12), Moderate (6-8), Mild (3-5), Normal (0-2)								
	Score of 3 or more for first 2 questions suggests anxiety								
	Score of 3 or more for last 2 questions suggests depression.								
	Total Score								
Mental State Exam ☐				Document in inpatient notes ☐					
Senior Medical Review ☐				Further investigations initiated ☐					
Psychiatry / Mental Health Referral ☐				Specify:					

2. General Management Principles

- Where appropriate/available, consider obtaining a collateral history from patient's family and/or carer, other members of the multidisciplinary team, other healthcare providers of the patient (e.g., General Practitioner, Nursing Home etc.).
- Engage family members / carer to help keep patient safe wherever appropriate/available.
- Discuss nursing management plans already in place for the patient, including any clinical companions/specials.
- Conduct necessary investigations using [Choosing Wisely](#) principles.
- Identify and treat underlying causes, discuss with a senior Medical Officer, if required.
- An assessment of a patient capacity and risk should be undertaken before the use of the doctrine of necessity and restrictive practices.
- The use of the least restrictive practice if deemed necessary to keep the patient and others free from harm and in consultation with the [SCGOPHCG Safe Use of Restrictive Practices in a Non- Authorised Setting](#).
- Communicate plans, including episodes of restrictive practice with family, next of kin, guardian and/or carers.
- Only use pharmacological intervention if reorientation or de-escalation techniques have failed, if there is risk to the safety of the patient, staff and/or others.

- Perform a mental state examination (MSE) prior to referral to the Consultation Psychiatry team and /or Mental Health Liaison Nurse.
- Psychology and Social Work are also available via E-referral.

3. Behaviour of Concern

If a patient has been identified on the PAAF as having mental health or behavioural risks continue to monitor for changes in their mental state. Refer to the nursing Behavioural Observation Form (BOF) for patterns of behaviour and consider use of the BoC escalation pathway to rule out organic causes. Remember that even in instances where a patient has demonstrated a steady baseline of behaviour there is always the risk that there may be acute mental state deterioration including delirium.

4. Delirium and/or Dementia

If delirium and/or dementia is highly suspected, refer to the [SCGOPHCG Cognitive Impairment Management \(Delirium and/or Dementia\) Clinical Practice Guideline](#) for management.

The organisation is required to monitor rates of delirium, the tools used for screening and multidisciplinary approach to caring for this cohort of patients. The SCGOPHCG guideline should be read in conjunction with the [ACSQHS Delirium Clinical Care Standard](#).

5. Mental Health Deterioration – Escalation and Management

It is crucial that if a patient is considered at severe risk of deterioration, harm to self or others, urgent discussion is recommended with:

- Psychiatric Liaison Nurse (PLN) for patients in the emergency department.
- Consultation Liaison Psychiatry Team (via switch) for inpatients during business hours.
- After-hours, the CAT team for initial review, followed by On-Call Psychiatrist for further advise:
 - Refer to the Emergency Department Psychiatry team if urgent review required.
 - Discuss the appropriateness of the use of Mental Health Act 2014.
 - Advise on any pharmacological management.

If the patient is considered low risk but there are suggestions of a mental illness driving behavioural deterioration (history of mental illness, psychiatry admissions, exhibiting multiple behaviours of concern) then an e-referral to Consultation Liaison Psychiatry Team should occur.

5.0 Post Escalation Management

1. Staff Debrief

Being exposed to any behavioural escalation can be confronting and over time contribute to psychological stress on staff members. Following incidents involving behavioural escalation, all persons involved, including the patient, who witnessed/involved in the incident should be offered [Psychological First Aid](#) and counselling where appropriate. Additional support options include.

- STOP: Take 5 & Talk (hot debrief) maybe asked for and delivered by the SRN 3+/line manager immediately after an incident.
- Timely and tailored support is essential in the reduction of negative outcomes of traumatic experiences. Further clinical/operational debriefing or ward/group sessions are a voluntary process, and an individual should decide whether they wish to engage.
- To clarify support options for staff, leaders may benefit from contacting [NMHS Wellbeing](#)

2. Patient Debrief

Patients can also be affected by the trauma of a behavioural escalation, restrictive practice, and it can influence the perception and possibly have an impact on future interactions with health services.

Patients should be offered a debriefing session post every behavioural escalation and restrictive practice episode. Patients have the right to refuse to engage in this process, please document the offer and acceptance or refusal in the inpatient notes.

In-reach services available to assist with patient debriefing are below and available via e-referral:

- Clinical Psychology
- Consultation Liaison Psychiatry
- Aboriginal Health Liaison Officers
- BERT Service

6.0 Discharge

Discharge planning should be in collaboration with the patient and their next of kin/family, guardian, and carer input. This could include management strategies that have been put in place during the patient's hospital stay, Allied Health and community-based referrals. BERT inpatient engagement plans (BEPs) can be shared with family, Residential Care Line and other relevant stakeholders on discharge. Please request BERT to complete a Discharge BEP.

7.0 Compliance, Monitoring and Evaluation

Compliance with this clinical practice guideline will be monitored and reported on quarterly to the Safety and Quality Standard 5 and 8 leads:

- Use PAAF and BOF for escalation of BD.
- The use of positive engagement management plans to assist during daily routines and escalations of behaviours.
- Use of BoC escalation pathway to escalate changes in cognition, behaviour, and mental health appropriately.
- Referral to appropriate in-reach services for management.
- Use Restrictive Practices across the SCGOPHCG, which includes pharmacological management.
- URGENT BERT, code black and restrictive practice monitoring.

Related Documents and SCGOPHCG Policy

- [Australian Commission on Safety and Quality in Health Care Delirium Clinical Care Standard](#)
- [National Safety and Quality Health Service \(NSQHS\) Standards Recognising and Responding to Acute Deterioration Standard](#)
- [National Safety and Quality Health Service \(NSQHS\) Standards Comprehensive Care](#)
- [National Consensus Statement: Essential elements for recognising and responding to deterioration in a person's mental state.](#)
- [NMHS Recognising and Responding to Acute Deterioration Policy](#)
- [SCGOPHCG Cognitive Impairment Management \(Delirium and/or Dementia\) Clinical Practice Guideline](#)
- [SCGOPHCG Detention of Patients and Discharge Against Medical Advice Policy](#)
- [SCGOPHCG Safe Use of Restrictive Practice in a Non-Authorised Setting](#)
- [SCGOPHCG Recognising and Responding to Acute Deterioration Procedure](#)

- [SCGOPHCG Behaviour Based Alert Policy](#)
- [Clinical Companion and Patient Special](#)

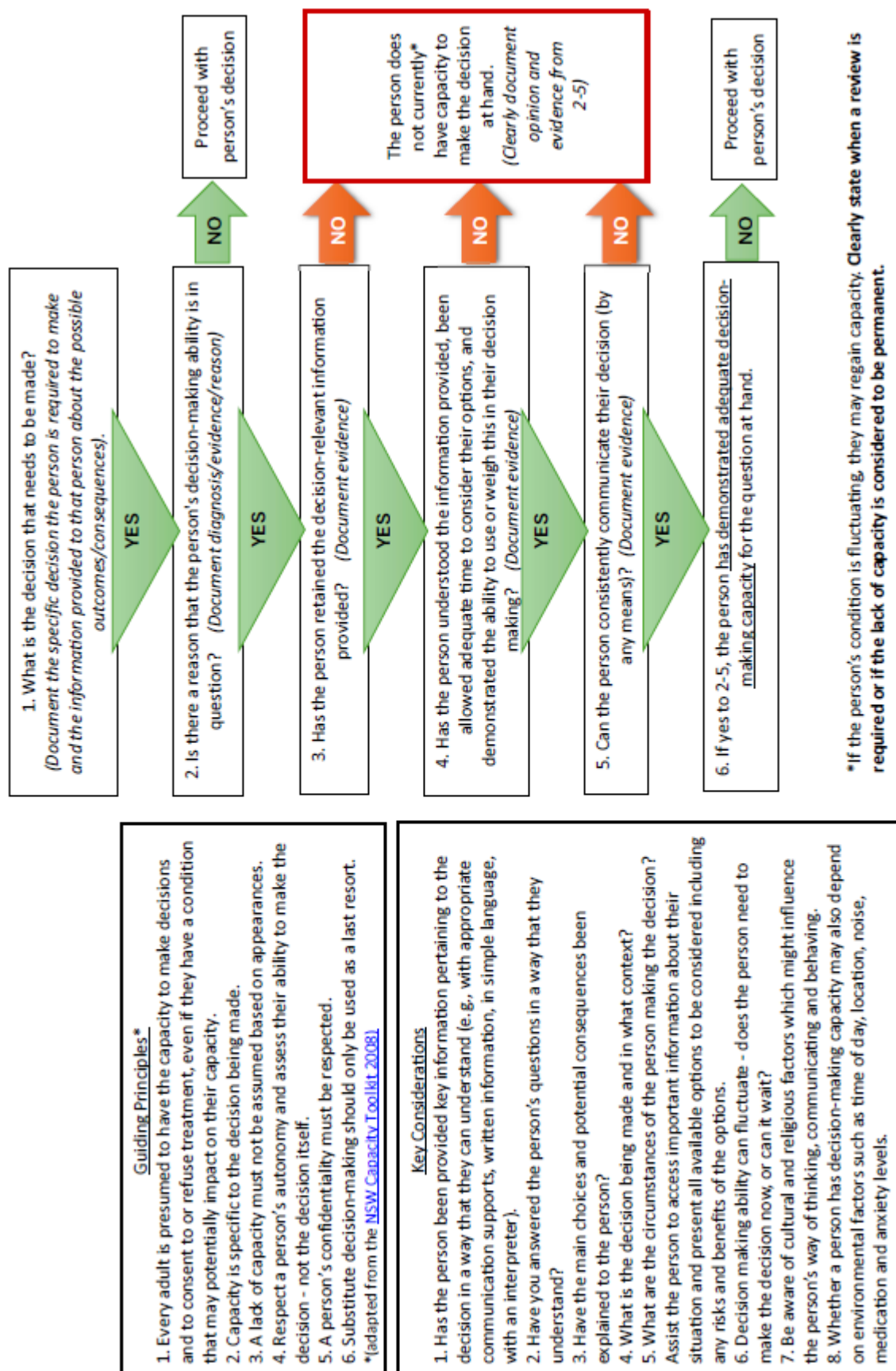
References

1. [Australian Commission on Safety and Quality in Health Care. Delirium Clinical Care Standards \[Internet\]. 2021](#) (cited 2023 April 17).
2. [4AT – Rapid Clinical Test for Delirium](#) [Internet].

Authorisation & Version Control

Policy Sponsor:	SCGOPHCG Co-Director Acute Services				
Policy Contact:	SCGOPHCG Coordinator of Nursing BERT				
Version #:	1.1	Last Reviewed:	18/12/2025	Review Due:	18/12/2028
Reference #	CPG007	Consumer Engagement:	N/A	Aboriginal Health ISD #:	4056
Approved by:	Co-Director Acute Services			Date:	19/02/2025
Endorsed by:	SCGOPHCG Clinical Practice Guideline Subcommittee			Date:	18/12/2025
Applicable Standards	NSQHS Standards: Std 1: Clinical Governance  Std 2: Partnering with Consumers  Std 4: Medication Safety  Std 5: Comprehensive Care  Std 6: Communicating for Safety  Std 8: Recognising and Responding to Acute Deterioration 				
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Appendix 1: Decision making Capacity Assessment Flow Chart



Appendix 2: Guideline for the Management of Acute Severe Behavioural Disturbances (Adults)

Guidelines for the Management of Acute Severe Behavioural Disturbances (Adults)

Pharmacotherapy is aimed at calming behaviour with the administration of a sedative medication. This should only be used when necessary and in conjunction with non-pharmacological management. Oral options are 1st line and alternative routes should only be used if an oral agent is not suitable. IM administration is reasonable when IV access is not available



In older adults, delirium and behavioral and psychological symptoms of dementia (BPSD) is common, these patients may demonstrate increased sensitivity to sedatives and antipsychotics, and the applicable guideline is:

Management of Agitation in Older Patients with Dementia or Delirium on Medical and Surgical Wards

In medically frail and/or high-risk patients*, consider dose adjustments:

Generally it would be reasonable to make an **initial dose reduction of 50%** to dose administered and to maximal dose.

In general, repeated subtherapeutic doses that are inadequate may result in prolonged distress, multiple drug administrations and cumulatively larger drug exposures.

Dosing intervals correspond to the typical time required for peak effect, dosing prior to these windows increases the risks from cumulative drug effect. If administered medication exceeds recommended maximal dose, senior advice should be sought.

These guidelines are provided to support clinicians with general recommendations and does not replace clinical judgement

Oral sedation

Lorazepam[#] 1 to 2 mg, if required, repeat in 30-60 mins, seek advice if >6mg in 24hrs
Diazepam 5 to 20mg, if required, repeat in 30-60 mins, seek advice if >60mg in 24hrs
Olanzapine 5 to 10mg, if required, repeat in 2-4 hours, seek advice if > 30mg in 24hrs

[#]Lorazepam is preferred to diazepam due to a shorter half-life and lower risk of accumulation

Rapid Sedation

Intramuscular (IM)

Droperidol 5 to 10mg IM, if required, repeat in 30-60 mins, seek advice if >20mg in 24hrs
Midazolam 5 to 10mg IM, if required, repeat in 30 mins, seek advice if >30mg in 24hrs
Olanzapine 5 to 10mg IM, if required, repeat in 2-4 hours, seek advice if >30mg in 24hrs

Combination therapy (Midazolam followed by Droperidol) may achieve more rapid sedation with reduced side effects¹

i.e: Midazolam 5mg IM + Droperidol 5mg IM

Intravenous (IV)

Droperidol 5 to 10mg IV, if required, repeat in 3-5 mins, seek advice if >20mg in 24hrs
Midazolam 2.5 to 5mg IV, if required, repeat in 3-5 mins, seek advice if >30mg in 24hrs

Combination therapy (Midazolam followed within 3-5 minutes by Droperidol) may achieve more rapid sedation with reduced side effects¹

i.e: Midazolam 2.5 to 5 mg IV + Droperidol 5 to 10mg IV

*IV sedation should only be administered by staff familiar with its use, typically higher doses are administered in smaller aliquots.



If inadequate response, senior input should be sought if escalation to ketamine is considered, outside of the ED / ICU setting, activation of a MET call for additional medical support would be appropriate

Rescue Sedation

Ketamine 1-2 mg/kg IV or 4-5 mg/kg IM, if required repeat in 10-20 mins²
Seek further advice if behaviour uncontrolled in excess of >200 mg ketamine

Common Drug Side Effects

Potential Side Effect	Drug Class			Comments
	Benzodiazepine	Antipsychotics	Ketamine	
Respiratory Depression	✓	✓	During rapid administration	Airway support, if related to benzodiazepine consider Flumazenil* 0.1-0.2 mg IV titrated to effect (Max 2mg) *Do not use in benzodiazepine dependent
Extrapyramidal Reactions		✓		Consider Bzotropine (0.02 mg/kg) Max 1 mg IM or IV, can repeat in 15 mins
QT Prolongation		✓	Avoid if possible	Consider cardiac monitoring and magnesium sulphate
Neuroleptic Malignant Syndrome		✓		Seek expert advice, use of Bromocriptine or Dantrolene may be considered

*Medically frail / high risk includes but is not limited to severe organ limiting disease, extremes of weight, obstructive sleep apnoea, known difficult airway, high aspiration risk, or previous known adverse reaction to sedation

Key References:

1) Taylor DM, et al. Midazolam-Droperidol, Droperidol, or Olanzapine for Acute Agitation: A Randomized Clinical Trial. Ann Emerg Med. 2017 Mar;69(3):318-326.

2) Isbister GK et al. Ketamine as Rescue Treatment for Difficult-to-Sedate Severe Acute Behavioral Disturbance in the Emergency Department. Ann Emerg Med. 2016 May;67(5):581-587

3) Faculty of Pain Medicine (ANZCA). Guideline for safe care for patients sedated in health care facilities for acute behavioural disturbance [PS63]. Melbourne: ANZCA; 2019

Sedation Assessment

A	Alert Awake, responsive
C	Confusion New onset or worsening
V	Voice Responds to verbal stimulus
P	Pain Responds to pain stimulus
U	Unresponsive No response to voice or pain

Monitoring

Recommended minimum monitoring following IM or IV³

Monitor depth of sedation using A(C)VPU tool

Vital Signs:
Measure at 5 mins, then every 15mins for 1hr, then every 30 mins for at least 2 hours
Oxygenation, continuous pulse oximetry unless ALERT

Capnography to monitor ventilation if available

12 Lead ECG:
Baseline as soon as practicable, assess QT interval

Positioning:
Left lateral and/or raise head of bed

Bladder Care:
Assess 2 hourly, consider bladder scan

Pressure Care:
Reposition 2 hourly, more frequent if high risk

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Appendix 3: Management of Agitation in Older Patients with Dementia or Delirium on Medical and Surgical Wards

This guide refers to patients with persistent behavioural problems such as

- Restlessness, fidgeting, jumpiness
- Pacing
- Pulling at lines and tubes
- Trying to leave ward or hospital
- Continually trying to get out of bed
- Shouting, swearing, arguing, insulting staff
- Talking to self
- Threatening or trying to hit, kick, bite or scratch

Behaviour like this is usually due to delirium or dementia. Remember...

- Delirium may indicate a serious underlying medical problem or medication side effect.
- Refer to Cognitive Impairment (CI) Medical Review document (MR887).
- Clinical Practice Guidelines: Cognitive Impairment Management on Share Point policy page.
- Agitated patients with delirium or dementia sometimes injure themselves or staff.
- Benzodiazepines usually make agitation worse in delirium and increase the risk of falls, aspiration etc.

This guide is not about managing patients with...

- Simple insomnia
- Simple threats to leave hospital against medical advice or conflict about a specific issue
- Behavioural problems due to a psychiatric disorder such as mania or psychosis

In these situations provide counselling or discuss with a senior colleague.

Step 1. Review diagnosis and screen for new problems such as

1

Dehydration
Electrolyte imbalance
Level of pain
Infection/sepsis/inflammation
Respiratory failure (hypoxia/hypercapnia)
Impaction of faeces
Urine retention
Medication toxicity: especially anticholinergics (e.g. tricyclics, oxybutynin, tiotropium), digoxin, sedatives/opioids, corticosteroids, anticonvulsants, antiparkinsons)

If withdrawal is suspected

- Use diazepam and thiamine
- Do not use antipsychotics
- Commence Alcohol Withdrawal Scale (AWS) and refer to AWS guidelines

Step 2. Assess Risk

Consider potential for violence, pulling out tubes, absconding, interference with other patients, falls, accidents etc.

If high risk

- Consider a single room
- Companion
- Discuss with senior clinician
- Consider medication

Step 3. Non-Pharmacological Interventions

- 3**
- Maintain low level sensory stimulation: soft sound & lighting
 - Single room if possible
 - Avoid multiple room changes and if required complete in daylight hours
 - Staff to engage patient calmly, distract and supervise
 - Avoid confrontation e.g. walk away, defer interventions or offer food and drink to diffuse tension
 - Consider 1:1 companion
 - Encourage family to stay and assist

Pharmacological Management of Agitation in Older Patients

Antipsychotic medications are not first line treatment and should be used with caution because of the association with strokes and cardiac events.
Please reference the High Risk Medicines Policy

Patient with parkinsonism: Use ONLY Quetiapine 12.5mg to 25mg (tablet) 4 hourly PRN. Max 50mg in 24 hours. DO NOT move onto step 4 or 5. If there is no response at this stage seek advice from a senior clinician.

Step 4. Use only ONE of these high potency antipsychotics.

4

These agents are calming without much sedation

1. **Risperidone** 0.25 to 0.5 mg (tablet or liquid) 4 hourly PRN Max. 2mg in 24 hours OR
2. **Haloperidol** 0.25 to 0.5 mg (tablet, liquid, IM) 4 hourly PRN Max. 2mg in 24 hours

If the response is inadequate escalate to senior staff

Upon senior review, proceed to Step 5

Step 5. Use only one of these more sedating antipsychotics

5

1. **Quetiapine** 12.5mg - 25mg (tablet) 4 hourly prn - Max 50mg in 24 hours OR
1. **Olanzapine** 2.5 mg (tablet) 4 hourly prn - Max 7.5mg in 24 hours

- ✓ Aim to use ONE drug and optimise first line
- ✓ Keep doses to a minimum: giving higher doses on the first night is unlikely to help
- ✓ If the situation is escalating get advice from a senior colleague
- ✓ Review prescription daily
- ✓ Refer to Department of Geriatric, Acute and Rehabilitation Medicine (GARM) or Older Adult Psychiatry during office hours

Step 6. Discharge Planning

6

Document management plan for de-escalation of antipsychotic or sedating medication in the medical record and on the discharge plan.

Appendix 4: Mental State Examination

MENTAL STATE EXAMINATION

Appearance- age, gender, race/ethnic background, build, hairstyle and colour, apparent health, level of hygiene, mode of dress, physical abnormalities.

Behaviour- eye contact, cooperativeness, motor activity, abnormal movements, expressive gestures.

Speech- articulation disturbances, rate (rapid, pressured, slow, retarded), volume (loud, quiet, whispered), quantity (poverty of speech, monotonous, mutism).

Mood/affect- mood (subjective); affect (objective) e.g. elevated, pressed, labile, angry, irritable, blunted, flattened, euphoric, incongruent, anxious; range and intensity, stability, appropriateness and continuity.

Thought stream- amount of speed of thought: poverty of thought, pressure of thought; slow or hesitant thinking.

Thought content- delusions of persecution; reference; delusions of control/influence/passivity; thought insertion; thought withdrawal; thought broadcasting. Other delusions not necessarily associated with schizophrenia: religious, nihilistic, morbid jealousy/infidelity, grandiose, guilt and worthlessness, somatic/hypochondriacal. Other: phobia, obsessions/compulsions, overvalued ideas.

MENTAL STATE EXAMINATION

Thought form- amount of thought and its production: poverty of ideas, flight of ideas; preservation (repetition of same thoughts), loosening of associations. Continuity of ideas: loosening of association/derailment, tangential, circumstantial, perseveration, thought blocking, concrete thinking, irrelevance. Disturbance in language: neologisms, incoherence/word salad, clanging.

Perception- hallucinations: auditory, visual, olfactory, gustatory, tactile. Depersonalisation; derealisation and illusions.

Cognition- level of consciousness/alertness; memory; orientation (time, place, person); concentration; abstract idea.

Insight- capacity to organise and understand problem, symptoms or illness; knowledge of medication; amenable to and compliance with treatment; impaired judgement.

Risks- summary of risks, including risks to self and others.

Impression- summary of signs and symptoms; consumers' strengths/weaknesses; supports; prior life experiences.

Plan- for ongoing management.

Reference: Adapted from Andrews G et al. Management of Mental Disorders: New Zealand Edition. 2000. Date of preparation: July 2013.
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